



Goebel Julia is the reporter for Team 15



Take over as reporter

**Your team has completed
100% of the test**



✓ Submit Test

Question 1 **Question Stem**

See attached Case Study. Which set of laboratory results is most consistent with a primary growth hormone excess?

 Question Attachment(s)

 CASE 1_GROWTH HORMONE_Endocrine Disorders TBL.pdf

Question 1 answer choices

- ☐ **A.** decreased IGF-1; increased GHRIH; increased GHRH
- ☐ **B.** decreased IGF-1; decreased GHRIH; increased GHRH
- ☐ **C.** increased IGF-1; decreased GHRIH; increased GHRH
- ☐ **D.** increased IGF-1; decreased GHRIH; decreased GHRH
- ☒ **E.** increased IGF-1; increased GHRIH; decreased GHRH

Answer Attachments(s)

+ Attachments

Do you have clarifications?



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the reporting role.

Question 2**Question Stem**

See attached Case Study. Which pathophysiological feature is most consistent with a patient diagnosed with acromegaly?

📎 Question Attachment(s)

📎 CASE 1_GROWTH HORMONE_Endocrine Disorders TBL.pdf

Question 2 answer choices

- ☐ A. abnormally tall stature
- ☒ B. hyperglycemia
- ☐ C. skeletal muscle atrophy
- ☐ D. hypotension
- ☐ E. obesity

Answer Attachments(s)

+ Attachments

Do you have clarifications?

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Question 3**Question Stem**

See attached Case Study. Which of the following is the correct treatment for pathology caused by an autosomal recessive mutation of the GH receptor?

** Question Attachment(s)**

 CASE 1_GROWTH HORMONE_Endocrine Disorders TBL.pdf

Question 3 answer choices

- ☒ **A. mecasermin**
- ☐ **B. sermorelin**
- ☐ **C. somatropin**
- ☐ **D. somatotropin**
- ☐ **E. somatostatin**

Answer Attachments(s)

+ Attachments

Do you have clarifications?

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misunderstood question

Question 4**Question Stem**

See attached Case Study. Which of the following agents is most likely to normalize IGF-1 level in a patient with high serum IGF-1 due to a pituitary tumor?

** Question Attachment(s)**

 CASE 1_GROWTH HORMONE_Endocrine Disorders TBL.pdf

Question 4 answer choices

- ☐ **A. Conivaptan**
- ☐ **B. Desmopressin**
- ☐ **C. Leuprolide**
- ☐ **D. Somatropin**
- ☒ **E. Octreotide**

Answer Attachments(s)

+ Attachments

Do you have clarifications?

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Question 5 **Question Stem**

Based on the attached Case Study, which of the following agents is most likely to cause hypoglycemia?

 Question Attachment(s)

 CASE 1_GROWTH HORMONE_Endocrine Disorders TBL.pdf

Question 5 answer choices

- ☐ **A.** Octreotide
- ☐ **B.** Conivaptan
- ☐ **C.** Pegvisomant
- ☐ **D.** Somatropin
- ☒ **E.** Mecasermin

Answer Attachments(s)

+ Attachments

Do you have clarifications?

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Question 6**Question Stem**

As described in the attached Case Study a significant feature of Bobbi's condition is widening of bones in the skull, hands, and feet. The endocrine hormone most directly responsible for this pathophysiological feature is primarily secreted from which tissue?

 Question Attachment(s)

 CASE 1_GROWTH HORMONE_Endocrine Disorders TBL.pdf

Question 6 answer choices

- ☐ **A.** anterior pituitary adenoma cells
- ☐ **B.** anterior pituitary somatotrophs
- ☐ **C.** bone and cartilage cells
- ☐ **D.** hypothalamic neuroendocrine cells
- ☒ **E.** liver endocrine cells

Answer Attachments(s)

+ Attachments

Do you have clarifications?



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Question 7**Question Stem**

See attached Case Study. What is the underlying pathophysiological mechanism causing abnormal frequent urination (polyuria)?

** Question Attachment(s)**

 CASE 1_GROWTH HORMONE_Endocrine Disorders TBL.pdf

Question 7 answer choices

- ☐ A. decreased plasma glucose uptake and utilization by tissues
- ☒ B. hyperglycemia
- ☒ C. increased adipose tissue lipolysis
- ☐ D. increased fatty acid utilization by tissues
- ☒ E. osmotic diuresis

Answer Attachments(s)

+ Attachments

Do you have clarifications?

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huh???

Question 8**Question Stem**

Based on the information in the attached Case Study, what is the underlying pathophysiological mechanism causing amenorrhea?

 Question Attachment(s)

 CASE 1_GROWTH HORMONE_Endocrine Disorders TBL.pdf

Question 8 answer choices

- ☐ A. adenoma growth encroachment on anterior pituitary gonadotrophs
- ☐ B. negative feedback inhibition of GnRH by prolactin
- ☐ C. adenoma secretion of growth hormone and prolactin
- ☒ D. complete occlusion of hypophyseal portal circulation by adenoma
- ☐ E. cross-stimulation of prolactin receptor by growth hormone excess

Answer Attachments(s)**+ Attachments**

Do you have clarifications?

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?????????

Question 9**Question Stem**

Based on the information on the attached Case Study, Bobbi was initially treated with bromocryptine and octreotide, which caused gastrointestinal side effects. If surgery had been contraindicated, Bobbi's physician might have offered another commonly used drug option that acts through which general mechanism of action?

 Question Attachment(s)

 CASE 1_GROWTH HORMONE_Endocrine Disorders TBL.pdf

Question 9 answer choices

- ☐ A. adenylyl cyclase stimulator
- ☐ B. osteoclast stimulator
- ☐ C. G-protein receptor agonist
- ☒ D. tyrosine-kinase signaling antagonist
- ☐ E. target cell hormone receptor down-regulation

Answer Attachments(s)**+ Attachments**

Do you have clarifications?



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Question 10**Question Stem**

See attached Case Study. What pharmacological properties give octreotide a therapeutic advantage over somatostatin? Choose the best answer

** Question Attachment(s)**

 CASE 1_GROWTH HORMONE_Endocrine Disorders TBL.pdf

Question 10 answer choices

- ☐ A. pulsatile exposure, longer half life, greater potency
- ☒ B. longer half life, greater potency
- ☐ C. pulsatile exposure, greater potency
- ☐ D. longer half life, greater potency, hits more therapeutic targets
- ☐ E. pulsatile exposure, longer half life

Answer Attachments(s)

+ Attachments

Do you have clarifications?



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Question 11**Question Stem**

See attached Case Study. If Bobbi experienced the clinical manifestations of this disorder starting at age 10 years how would the clinical presentation differ?

 Question Attachment(s)

 CASE 1_GROWTH HORMONE_Endocrine Disorders TBL.pdf

Question 11 answer choices

- ☐ A. More prominent headaches
- ☒ B. More prominent vertical growth
- ☐ C. More prominent excess body weight
- ☐ D. Shorter stature
- ☐ E. Worsened hyperglycemia

Answer Attachments(s)

+ Attachments

Do you have clarifications?

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Question 12**Question Stem**

See attached Case Study. What effect do glucocorticoids have on the following immune parameters?

** Question Attachment(s)**

 CASE 2_ADRENAL HORMONES_Endocrine Disorders TBL.pdf

Question 12 answer choices

- ☐ **A.** increased IL-1, increased IL-10, activation of macrophage apoptosis, increase in lymphocytes
- ☐ **B.** increased IL-1, increased IL-10, suppression of macrophage apoptosis, decrease in lymphocytes
- ☒ **C.** decreased IL-1, increased IL-10, activation of macrophage apoptosis, decrease in lymphocytes
- ☐ **D.** decreased IL-1, decreased IL-10, activation of macrophage apoptosis, increase in lymphocytes

- ☐ E. increased IL-1, decreased IL-10,
suppression of macrophage apoptosis,
decreased in lymphocytes

Answer Attachments(s)

+ Attachments

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Question 13**Question Stem**

See attached Case Study. Which of the following has the LONGEST half-life and GREATEST potency with respect to the glucocorticoid receptor?

 Question Attachment(s)

 CASE 2_ADRENAL HORMONES_Endocrine Disorders TBL.pdf

Question 13 answer choices

- ☒ **A. dexamethasone**
- ☐ **B. methylprednisone**
- ☐ **C. fludrocortisone**
- ☐ **D. fluticasone**
- ☐ **E. prednisolone**

Answer Attachments(s)

+ Attachments

Do you have clarifications?

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Question 14**Question Stem**

See attached Case Study. Licorice extract, used for flavoring candy, contains glycyrrhizic acid, an effective inhibitor of 11-beta-HSD type II. What serious effect can result from ingesting too much naturally flavored licorice?

 Question Attachment(s)

 CASE 2_ADRENAL HORMONES_Endocrine Disorders TBL.pdf

Question 14 answer choices

- ☐ **A.** glucocorticoid insufficiency
- ☐ **B.** myxedema
- ☐ **C.** hypoglycemia
- ☐ **D.** thyroid storm
- ☒ **E.** hypertensive crisis

Answer Attachments(s)[+ Attachments](#)**Do you have clarifications?**



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Question 15**Question Stem**

See attached Case Study. What effect would you expect an ACE inhibitor to have on circulating plasma levels of cortisol and aldosterone?

** Question Attachment(s)**

 CASE 2_ADRENAL HORMONES_Endocrine Disorders TBL.pdf

Question 15 answer choices

- ☐ A. decreased cortisol, decreased aldosterone
- ☒ B. no effect on cortisol, decreased aldosterone
- ☐ C. increased cortisol, no effect on aldosterone
- ☐ D. no effect on cortisol, no effect on aldosterone
- ☐ E. increased cortisol, decreased aldosterone

Answer Attachments(s)

+ Attachments

Do you have clarifications?



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Question 16**Question Stem**

See attached Case Study. Which of the following clinical features is most consistent with hyperglucocorticoid conditions?

** Question Attachment(s)**

 CASE 2_ADRENAL HORMONES_Endocrine Disorders TBL.pdf

Question 16 answer choices

- ☐ A. hypoglycemia
- ☐ B. hypotension
- ☒ C. hypokalemia
- ☐ D. hypovolemia
- ☐ E. hyponatremia

Answer Attachments(s)

+ Attachments

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Question 17**Question Stem**

See attached Case Study. Before the CT scan, a HIGH-dose dexamethasone test was also performed. What result would most likely suggest a mass in the adrenal gland?

For discussion: Why does result occur when an adenoma is present? What would this test show in the absence of pathology?

 Question Attachment(s)

 CASE 2_ADRENAL HORMONES_Endocrine Disorders TBL.pdf

Question 17 answer choices

- ☐ A. High ACTH, High Cortisol
- ☒ B. Low ACTH, High Cortisol
- ☐ C. High ACTH, Low Cortisol
- ☐ D. Low ACTH, Low Cortisol

Answer Attachments(s)

+ Attachments

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Question 18**Question Stem**

See attached Case Study. Which of the following is the underlying pathology that causes Harold's high blood pressure (165/105)?

📎 Question Attachment(s)

📄 CASE 2_ADRENAL HORMONES_Endocrine Disorders TBL.pdf

Question 18 answer choices

- ☐ A. Hyperglycemia
- ☐ B. Hypernatremia
- ☒ C. Elevated glucocorticoids
- ☐ D. Increased vascular fluid volume
- ☐ E. Up-regulated vascular smooth muscle beta₁-adrenergic receptors

Answer Attachments(s)

+ Attachments

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Question 19**Question Stem**

See attached Case Study. Regarding the previous question, what is the pathophysiological mechanism by which this underlying pathology causes elevated blood pressure?

** Question Attachment(s)**

 CASE 2_ADRENAL HORMONES_Endocrine Disorders TBL.pdf

Question 19 answer choices

- ☐ **A.** Causes abnormal production of corticosterone and 11-deoxycorticosterone
- ☐ **B.** Decreases catecholamine sensitivity of vascular smooth muscle
- ☐ **C.** Increases secretion of adrenal epinephrine
- ☒ **D.** Stimulates target tissue aldosterone receptors
- ☐ **E.** Stimulates renin and angiotensin production

Answer Attachments(s)

+ Attachments

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Question 20**Question Stem**

See attached Case Study. If surgery was delayed, one possibility was that Harold could be treated with a drug to suppress cortisol biosynthesis, such as ketoconazole. However, this was not done due to specificity concerns based on its mechanism of action. Using the attached Steroid Hormone Biosynthesis figure, which human P450 enzyme is inhibited by ketoconazole? For discussion: what are the specificity concerns?

📎 Question Attachment(s)

Steroid Biosynthesis Pathway.jpg

(<https://cdn.filestackcontent.com/XnJj5lQmacpaeMPX6FA>)

CASE 2_ADRENAL HORMONES_Endocrine Disorders TBL.pdf

Question 20 answer choices

☐ A. 11-OH

☒ B. 17-OH

☒ C. 21-OH

☒ D. 20,22 Des

☐ E. 3b-HSD

Answer Attachments(s)

+ Attachments

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Question 21**Question Stem**

See attached Case Study. Why was Harold's ACTH reading so low? What pharmacological state mimics this pathophysiological condition?

 Question Attachment(s)

 CASE 2_ADRENAL HORMONES_Endocrine Disorders TBL.pdf

Question 21 answer choices

- ☐ **A.** Low CRH fails to stimulate sufficient ACTH release; corticosteroid rebound
- ☐ **B.** Low GHRH fails to stimulate sufficient ACTH release; iatrogenic diabetes insipidus
- ☐ **C.** Low cortisol exerts feedback on the anterior pituitary, suppressing ACTH; iatrogenic Cushing's syndrome
- ☒ **D.** High cortisol exerts negative feedback on the anterior pituitary, suppressing ACTH release; iatrogenic Cushing's syndrome
- ☐ **E.** high CRH negatively affects ACTH release from the anterior pituitary; iatrogenic Addison's

Answer Attachments(s)

+ Attachments

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Question 22**Question Stem**

See attached Case Study. If Harold's ACTH was elevated in this case presentation (instead of suppressed) what would be your next step in the clinical investigation?

** Question Attachment(s)**

 CASE 2_ADRENAL HORMONES_Endocrine Disorders TBL.pdf

Question 22 answer choices

- ☐ A. Adrenal biopsy
- ☐ B. Adrenal CT/MRI
- ☒ C. Pituitary MRI
- ☐ D. Trans-sphenoidal biopsy/removal
- ☐ E. Ultrasound MRI

Answer Attachments(s)

+ Attachments

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Question 23**Question Stem**

See attached Case Study. What is the most common etiology of the condition that Harold is presenting with?

** Question Attachment(s)**

 CASE 2_ADRENAL HORMONES_Endocrine Disorders TBL.pdf

Question 23 answer choices

- ☐ A. Adrenal mass
- ☐ B. Adrenal hyperplasia
- ☒ C. Iatrogenic steroid use
- ☐ D. Pituitary adenoma
- ☐ E. Hypothalamic mass

Answer Attachments(s)

+ Attachments

Do you have clarifications?

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Question 24**Question Stem**

See attached Case Study. What two-word answer below best describes the MOA of thyroid hormones?

** Question Attachment(s)**

 CASE 3_THYROID HORMONES_Endocrine Disorders TBL.pdf

Question 24 answer choices

- ☐ A. AC activation
- ☐ B. PIP/IP3 activation
- ☐ C. iodine sequestration
- ☒ D. transcriptional regulation
- ☐ E. thermohomeostasis regulation

Answer Attachments(s)

+ Attachments

Do you have clarifications?

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Question 25**Question Stem**

See attached Case Study. What explains the paradoxical effect of iodide administration to decrease thyroid hormone production? For discussion: describe and explain this effect.

** Question Attachment(s)**

 CASE 3_THYROID HORMONES_Endocrine Disorders TBL.pdf

Question 25 answer choices

- ☐ **A.** acid-base effects
- ☐ **B.** Le Chatelier's Principle
- ☐ **C.** the Jod-Basedow effect
- ☒ **D.** the Wolff-Chaikoff effect
- ☐ **E.** the Watson-Crick effect

Answer Attachments(s)

+ Attachments

Do you have clarifications?

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Question 26**Question Stem**

See attached Case Study. What significant drawback limits the current use of dessicated thyroid extract to treat hypothyroidism? Choose the best answer:

 Question Attachment(s)

 CASE 3_THYROID HORMONES_Endocrine Disorders TBL.pdf

Question 26 answer choices

- ☒ **A.** possible biological contaminants
- ☐ **B.** high T3 content
- ☐ **C.** slow onset
- ☐ **D.** non-specific binding targets
- ☐ **E.** allergic reaction

Answer Attachments(s)

+ Attachments

Do you have clarifications?

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Question 27**Question Stem**

See attached Case Study. What thyroid derangement would benefit from corticosteroid administration, and by what MOA?

** Question Attachment(s)**

 CASE 3_THYROID HORMONES_Endocrine Disorders TBL.pdf

Question 27 answer choices

- ☐ A. hypothyroidism, stimulating T4 production
- ☒ B. hyperthyroidism, suppressing TSH
- ☐ C. hypothyroidism, stimulating TRH
- ☐ D. hyperthyroidism, suppressing TRH
- ☐ E. hypothyroidism, suppressing TSH

Answer Attachments(s)

+ Attachments

Do you have clarifications?

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Question 28**Question Stem**

See attached Case Study. Regarding the cellular processes required to biosynthesize thyroid hormones, uptake of iodide (I^-) from blood into thyroid follicular cells involves

 Question Attachment(s)

 CASE 3_THYROID HORMONES_Endocrine Disorders TBL.pdf

Question 28 answer choices

- ☐ **A.** exchange of intracellular K^+ for extracellular iodide
- ☐ **B.** facilitated diffusion of iodide from blood into follicular cells
- ☐ **C.** primary active transport of iodide from blood into follicular cell
- ☐ **D.** direct regulation by Thyroid Peroxidase (TPO)
- ☒ **E.** co-transport of extracellular Na^+ and iodide into follicular cells

Answer Attachments(s)**+ Attachments**

Do you have clarifications?



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Question 29**Question Stem**

See attached Case Study. You are asked to evaluate a 35 year-old male patient. The only clinical information that is given is that he has developed a significant goiter. Based on other observed clinical manifestations, you suspect some type of thyroid hormone related disorder and request a plasma thyroid hormone &TSH analysis. Results were as follows:

- Total T4 = 28 $\mu\text{g/dL}$ (Normal Range = 5-12 $\mu\text{g/dL}$)
- Total T3 = 325 ng/dL (Normal Range =80-200 ng/dL)
- TSH = 0.15 mIU/ml (Normal Range = 0.35-7.0 mIU/ml)

Laboratory data is most consistent with which disorder?

 Question Attachment(s)

 CASE 3_THYROID HORMONES_Endocrine Disorders TBL.pdf

Question 29 answer choices

- ☐ A. blockage of hypothalamic-pituitary portal circulation
- ☐ B. chronic iodine deficiency
- ☒ C. Graves' disease

- ☐ **D.** Hashimoto's thyroiditis
- ☐ **E.** hypersecreting thyrotroph adenoma

Answer Attachments(s)

+ Attachments

Do you have clarifications?

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Question 30**Question Stem**

See attached Case Study. Which of the following is a pathophysiological feature of Graves' disease that distinguishes this disorder from hypothyroid conditions?

📎 Question Attachment(s)

📎 CASE 3_THYROID HORMONES_Endocrine Disorders TBL.pdf

Question 30 answer choices

- ☐ A. goiter
- ☐ B. myxedema
- ☒ C. exophthalmos
- ☐ D. skeletal muscle atrophy
- ☐ E. autoimmune etiology

Answer Attachments(s)

+ Attachments

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Question 31**Question Stem**

See attached Case Study. Among potential etiologies, Natasha's physician considered the possibility that she might be ingesting exogenous thyroxine (T4). However, he eliminated this etiology based on physical findings and laboratory testing. Which of the following most definitively supported by his conclusion?

 Question Attachment(s)

 CASE 3_THYROID HORMONES_Endocrine Disorders TBL.pdf

Question 31 answer choices

- ☐ **A.** serum TSH level
- ☐ **B.** serum TRH and TSH levels
- ☒ **C.** thyroid radioactive I⁻ uptake
- ☐ **D.** Natasha's wide-eyed stare and full lower neck
- ☒ **E.** free T3/T4 (ratio)

Answer Attachments(s)**+ Attachments**

Do you have clarifications?



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Question 32**Question Stem**

See attached Case Study. What is the underlying pathophysiological mechanism related to Natasha's hyperthyroid state that causes the symptom of increased arterial blood pressure?

 Question Attachment(s)

 CASE 3_THYROID HORMONES_Endocrine Disorders TBL.pdf

Question 32 answer choices

- ☐ **A.** increased cardiac output
- ☐ **B.** increased vascular smooth muscle vasoconstriction
- ☒ **C.** increased catecholamine sensitivity
- ☐ **D.** increased pulse pressure
- ☐ **E.** increased cardiac contractility

Answer Attachments(s)

+ Attachments

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Question 33**Question Stem**

See attached Case Study. Given this person's age and medical history what initial treatment recommendation do you have for this patient? For discussion: if you choose "medication therapy", describe and justify.

 Question Attachment(s)

 CASE 3_THYROID HORMONES_Endocrine Disorders TBL.pdf

Question 33 answer choices

- ☐ A. Medication therapy for life
- ☒ B. Medication therapy and serial monitoring
- ☐ C. Radioactive iodine ablation
- ☐ D. Thyroidectomy
- ☐ E. Thyroxine

Answer Attachments(s)

+ Attachments

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Question 34**Question Stem**

See attached Case Study. After 1 month of treatment, Natasha returns with clinical symptoms suggestive of thyroid storm. She is subsequently treated with 10 mg methimazole three times daily for two months, followed six days later by ^{131}I (14.5mCi). What is the most likely rationale for this treatment regimen?

 Question Attachment(s)

 CASE 3_THYROID HORMONES_Endocrine Disorders TBL.pdf

Question 34 answer choices

- ☒ **A.** Methimazole was used to lower the thyroid hormone before radioiodine therapy
- ☒ **B.** Radioiodine therapy alone may lead to increased free T4 before hypothyroidism
- ☐ **C.** Methimazole alone is ineffective in treating her disease
- ☒ **D.** Methimazole treatment helps to reduce the dosage of radioiodine

- ☐ E. Radioiodine was used to avoid long term use of anti-thyroid drugs

Answer Attachments(s)

[+ Attachments](#)

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Question 35**Question Stem**

See attached Case Study. Natasha is most worried about the eye changes that come with this condition. Which of the following statements are true?

 Question Attachment(s)

 CASE 3_THYROID HORMONES_Endocrine Disorders TBL.pdf

Question 35 answer choices

- ☐ **A.** She has been diagnosed and with treatment the eye changes will improve now
- ☐ **B.** The eye changes always precede the biochemical changes
- ☐ **C.** The eye changes always follow the biochemical changes
- ☐ **D.** Treatment will increase the risk of eye changes
- ☒ **E.** The eye changes can occur anytime over the course of the disease and are independent of treatment

Answer Attachments(s)

+ Attachments

Do you have clarifications?

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Question 36**Question Stem**

See attached Case Study. Natasha would like to know what to expect over time with her treatment. Which of the following statements is correct?

** Question Attachment(s)**

 CASE 3_THYROID HORMONES_Endocrine Disorders TBL.pdf

Question 36 answer choices

- ☐ **A.** All patients will require medication to suppress the thyroid for their lifetime
- ☐ **B.** Most patients will need radioactive iodine to be able to take no thyroid medications
- ☐ **C.** Surgery is the only way to assure a medication free life
- ☒ **D.** Up to 30% of people with Graves' disease will burn out and may not need any medication long term
- ☐ **E.** Natasha's offspring will need treatment if surgery is selected

Answer Attachments(s)

+ Attachments

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